PharmEasy Lab

Patient Name	: Mr.SHYAM SINGH	Barcode	: E00761462
Age/Gender	: 82 Y 0 M 1 D/M	Sample Collected On	: 07/Jun/2021 08:18AM
Order Id	: 8873172	Sample Received On	: 09/Jun/2021 08:44AM
Referred By	:	Report Generated On	: 09/Jun/2021 10:27AM
Sample Type	: WHOLE BLOOD EDTA	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF HEMATOLOGY

COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
COMPLETE BLOOD COUNT (CBC) , *WHOLE BLOOD EDTA*			
HAEMOGLOBIN Spectrophotometer	12.8	g/dL	13-17
RBC COUNT Electrical Impedence	4.68	Million/cu.mm	4.5-5.5
PCV Electronic pulse & Calculation	39.00	%	40-50
MCV Calculated	83	fL	83-101
MCH Calculated	27.3	pg	27-32
MCHC Calculated	32.7	g/dL	31.5-34.5
R.D.W-CV Calculated	14.5	%	11.6-14
R.D.W-SD Calculated	43.00	fl	37.0 - 54.0
PLATELET COUNT Electrical impedence	204000	Cells/cu.mm	150000-410000
TOTAL WBC COUNT Electrical Impedance	10,200	Cells/cu.mm	4000-10000
DIFFERENTIAL LEUCOCYTIC COUNT (DLC) Electrical Impedance			
NEUTROPHILS %	54.6	%	40-80
LYMPHOCYTES %	33.4	%	20-40

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DEPARTMENT OF HEMATOLOGY

COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval

PERIPHERAL SMEAR , *WHOLE BLOOD EDTA*

PERIPHERAL SMEAR

RBC'S ARE NORMOCYTIC NORMOCHROMIC
WBC'S ARE NORMAL IN TOTAL NUMBER , MORPHOLOGY AND DISTRIBUTION. NO IMMATURE CELLS SEEN.
PLATELETS ARE ADEQUATE.
NO HEMOPARASITES SEEN.

IMPRESSION: NORMOCYTIC NORMOCHROMIC BLOOD PICTURE.

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Age/Gender	: 82 Y 0 M 1 D/M	Sample Collected On	: 07/Jun/2021 08:18AM
Order Id	: 8873172	Sample Received On	: 09/Jun/2021 08:40AM
Referred By	:	Report Generated On	: 09/Jun/2021 12:08PM
Sample Type	: NAF PLASMA	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY

COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
GLUCOSE, FASTING , *NAF PLASMA*			
GLUCOSE, FASTING GOD - POD	133	mg/dL	70 - 100

Americal Diabetes Association (ADA) guideliness Blood Sugar Levels for diagnosing Diabetes and Prediabetes

Condition	Fasting Blood Glucose
Normal	<100
Pre-Diabetes	100-125
Diabetes	More than 126*

* A confirmatory test of either a fasting blood sugar, OGTT or random blood sugar with symptoms must be done on another day (Except in the case of unequivocal hyperglycemia with metabloic decompensation)

*Atleast 8-10 hours fasting is mandatory for Fasting Blood Glucose/Sugar. If not, values might fluctuate.

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Order Id	: 8873172	Sample Received On	: 09/Jun/2021 08:44AM
Referred By	:	Report Generated On	: 09/Jun/2021 12:59PM
Sample Type	: WHOLE BLOOD EDTA	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
HBA1C; GLYCOSYLATED HEMOGLOBIN , *WHOLE BLOOD EDTA*			
HbA1c, GLYCATED HEMOGLOBIN HPLC	7.8	%	
ESTIMATED AVERAGE GLUCOSE (eAG) Calculated	177	mg/dL	

Kindly Correlate Clinically.

General Information's

REFERENCE GROUP	HbA1c IN %
NON DIABETIC ADULTS >18 YEARS	<5.7
AT RISK (PREDIABETES)	5.7 – 6.4
DIAGNOSING DIABETES	>= 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Reference Range as per American Diabetes Association (ADA)
Note: Dietary preparation or fasting is not required.
1. A1C test should be performed at least two times a year in patients who are meeting treatment goals (and who have stable glycemic control).

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Sample Type	: SERUM	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
LIPID PROFILE (7 PARAMETERS) , *SERUM*			
TOTAL CHOLESTEROL CHE/CHO/POD	115	mg/dL	Desirable:<200 Borderline High:200-239 High:>240
TRIGLYCERIDES Enzymatic	80	mg/dL	Normal:<150 Borderline High:151-199 High:200-499 Very High:>500
HDL CHOLESTEROL CHE/CHO/POD	46	mg/dL	High (Desirable):>60 Acceptable:40-59 Low:<39
NON-HDL CHOLESTEROL Calculated	68.80	mg/dl	Optimal:<130 Near&Above optimal: 131-159 Borderline High:160-189 High: 190-219 Very high: >220
LDL CHOLESTEROL Calculated	52.8	mg/dL	Optimal:<100 Near Optimal:101-129 Borderline High:130-159 High:160-189 Very High:>190
VLDL CHOLESTEROL Calculated	16	mg/dL	<30
CHOL / HDL RATIO Calculated	2.49	Ratio	0-4.97

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Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
RENAL PROFILE (6 PARAMETERS) , *SERUM*			
UREA Urease	24.00	mg/dL	19.0-43.0
BLOOD UREA NITROGEN Calculated	11.2	mg/dL	9.0 - 20.0
CREATININE Amidohydrolase	0.76	mg/dL	0.66 - 1.25
UREA/CREATININE RATIO Calculated	31.58	Ratio	
BUN / CREATININE RATIO Calculated	14.74	Ratio	
URIC ACID Uricase	3.40	mg/dL	3.5-8.5

General Information:

Preanalytical issues such as high-protein intake and increased muscle bulk may lead to elevated creatinine levels not representative of actual renal function in an individual. Likewise, serum creatinine as a marker of renal function is often unreliable in the those with decreased muscle bulk such as the elderly, amputees and is individuals affected by muscular dystrophy.

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Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
LIVER FUNCTION TEST (11 PARAMETERS) , *SERUM*			
BILIRUBIN-TOTAL Diazonium salt	0.28	mg/dL	0.20 - 1.20
BILIRUBIN-DIRECT Spectrophotometric	0.10	mg/dL	0.0-0.3
BILIRUBIN-INDIRECT Direct measure	0.18	mg/dL	0.0-1.10
ALKALINE PHOSPHATASE PNP	78.00	U/L	38-126
AST/SGOT UV with P-5-P	35.0	U/L	17-59
ALT/SGPT UV with P-5-P	28.0	U/L	<50.0
AST:ALT RATIO Calculated	1.25	Ratio	Upto 1.3
PROTEIN, TOTAL Biuret	7.40	g/dL	6.3-8.2
ALBUMIN Bromocresol Green	4.10	g/dL	3.5 - 5.0
GLOBULIN Calculated	3.30	g/dL	2.0-3.5
A/G RATIO Calculated	1.24		0.8-2.0

Genral Information:

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Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
IRON PROFILE , *SERUM*			
IRON Ferene	57.0	µg/dL	49-181
TIBC - d Dye Binding	334	µg/dL	261-462
% OF SATURATION Calculated	17.07	%	14-50
UIBC - c Calculated	277.00	µg/dL	135 - 392

Transferrin is the primary plasma iron transport protein, which binds iron strongly at physiological pH. Transferrin is generally only 25% to 30% saturated with iron. The additional amount of iron that can be bound is the unsaturated iron-binding capacity (UIBC). Diurnal variation is seen in serum iron levels—normal values in midmorning, low values in midafternoon, very low values (approximately 10 µg/dL) near midnight.

TIBC measures the blood's capacity to bind iron with transferrin (TRF). Estrogens and oral contraceptives increase TIBC levels. Asparaginase, chloramphenicol, corticotropin, cortisone, and testosterone decrease the TIBC levels.

% saturation represents the amount of iron-binding sites that are occupied. Iron saturation is a better index of iron stores than serum iron alone. % saturation is decreased in iron deficiency anemia (usually <10% in established deficiency).

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DEPARTMENT OF BIOCHEMISTRY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
EGFR - ESTIMATED GLOMERULAR FILTRATION RATE , *SERUM*			
CREATININE Amidohydrolase	0.76	mg/dL	0.66 - 1.25
EGFR - CREATININE AND GLOMERULAR FILTRATION RATE CKD-EPI FORMULA	84.96	mL/min/1.73m²	

Stage	e-GFR	Description
1	90+	Normal kidney function but urine findings or structurel abnormalities or genetic trait point to kidney disease
2	60 to 89	Mildly reduced kidney function and other findings (as for stage 1) point to kideny disease
3A	45 to 59	Moderate reduced kidney function
3B	30 to 44	Moderate reduced kidney function
4	15 to 29	Severely reduced kidney function
5	<15 or on dialysis	Very severe or end stage kidney failure

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Referred By	:	Report Generated On	: 09/Jun/2021 02:28PM
Sample Type	: SERUM	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF IMMUNOLOGY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
THYROID PROFILE (TOTAL T3, TOTAL T4, TSH) , *SERUM*			
TRI-IODOTHYRONINE (T3, TOTAL) ECLIA	1.52	ng/mL	0.97-1.80
THYROXINE (T4, TOTAL) ECLIA	12.40	µg/dL	5.53-14.00
TSH ECLIA	1.690	µIU/mL	0.46-4.68

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences.TSH levels are influenced by circadian rhythm, reaches peak level between 2 to 4 am and at its minimum between 6 to 10pm. hence time of the day significantly affects serum TSH concentrations.

Important note for the people with borderline TSH levels (4.0 to 10 mIU/L).
People with borderline TSH level while have no or mild to moderate sign & symptoms of thyroid issues, are considered to have **Sub-Clinical Hypothyroidism (SCH)**. Some time it could be a temporary fluctuation due to many reasons like age factor, circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies, and a retest (after 3 or 4 weeks) may be needed along with the test **Free T4** for further evaluation and confirmation.

For pregnant females	**Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)**
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0

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Order Id	: 8873172	Sample Received On	: 09/Jun/2021 10:11AM
Referred By	:	Report Generated On	: 09/Jun/2021 12:43PM
Sample Type	: URINE	Sample Temperature	: Maintained
Visit No.	: MED001.234076	Rate Type	: PHARMEASY LAB

DEPARTMENT OF CLINICAL PATHOLOGY
COMPREHENSIVE FULL BODY CHECKUP

Test Name	Result	Unit	Bio. Ref. Interval
COMPLETE URINE EXAMINATION , *URINE*			
PHYSICAL EXAMINATION			
VOLUME	15.00	mL	
COLOUR Visual	YELLOW		PALE YELLOW
TRANSPARENCY Visual	TURBID		CLEAR
pH Dipstick	9.0		5-7.5
SP. GRAVITY Dipstick	1.015		1.002-1.030
BIOCHEMICAL EXAMINATION Dipstick/Benedicts Test			
URINE PROTEIN	NEGATIVE		NEGATIVE
GLUCOSE	NEGATIVE		NEGATIVE
URINE BILIRUBIN	NEGATIVE		NEGATIVE
KETONES	NEGATIVE		NEGATIVE
UROBILINOGEN	NORMAL		NORMAL
BLOOD	NEGATIVE		NEGATIVE
NITRITE	NEGATIVE		NEGATIVE
MICROSCOPIC EXAMINATION			

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